

Senate File 383

H-1302

1 Amend Senate File 383, as amended, passed, and reprinted by
2 the Senate, as follows:

3 1. By striking everything after the enacting clause and
4 inserting:

5 <DIVISION I

6 PHARMACY BENEFITS MANAGERS

7 Section 1. Section 510B.1, Code 2025, is amended by adding
8 the following new subsections:

9 NEW SUBSECTION. 11A. "*National average drug acquisition*
10 *cost*" means the monthly survey of retail pharmacies conducted
11 by the federal centers for Medicare and Medicaid services
12 to determine average acquisition cost for Medicaid covered
13 outpatient drugs.

14 NEW SUBSECTION. 11B. "*Pass-through pricing*" means a
15 model of prescription drug pricing in which payments made
16 by a third-party payor to a pharmacy benefits manager for
17 prescription drugs are equivalent to the payments the pharmacy
18 benefits manager makes to the dispensing pharmacy or dispensing
19 health care provider for the prescription drugs, including any
20 professional dispensing fee.

21 NEW SUBSECTION. 21A. "*Specialty drug*" means a drug used
22 to treat chronic and complex, or rare medical conditions and
23 that requires special handling or administration, provider care
24 coordination, or patient education that cannot be provided by a
25 nonspecialty pharmacy or pharmacist.

26 NEW SUBSECTION. 21B. "*Spread pricing*" means a pharmacy
27 benefits manager charges a third-party payor more for
28 prescription drugs dispensed to a covered person than the
29 amount the pharmacy benefits manager reimburses the pharmacy
30 for dispensing the prescription drugs to a covered person.

31 NEW SUBSECTION. 22A. "*Wholesale acquisition cost*" means the
32 same as defined in 42 U.S.C. §1395w-3a(c)(6)(B).

33 Sec. 2. NEW SECTION. 510B.4B Prohibited conduct.

34 1. A pharmacy benefits manager shall not require a covered
35 person, as a condition of payment or reimbursement, to purchase

1 pharmacy services, including prescription drugs, exclusively
2 through a mail order pharmacy. This subsection shall not apply
3 to a specialty drug.

4 2. A pharmacy benefits manager shall not unreasonably
5 designate a prescription drug as a specialty drug to prevent
6 a covered person from accessing the prescription drug, or
7 limiting a covered person's access to the prescription drug,
8 from a pharmacy or pharmacist that is within the health
9 carrier's network. A covered person or pharmacy harmed by an
10 alleged violation of this subsection may file a complaint with
11 the commissioner and the commissioner shall, in consultation
12 with the board of pharmacy, make a determination if the covered
13 prescription drug meets the definition of specialty drug.

14 Sec. 3. Section 510B.8, Code 2025, is amended by adding the
15 following new subsection:

16 NEW SUBSECTION. 3. For the purpose of reducing premiums,
17 one hundred percent of all rebates received by a pharmacy
18 benefits manager shall be passed through to the health carrier,
19 or to the employee plan sponsor as permitted by the federal
20 Employee Retirement Income Security Act of 1974, 29 U.S.C.
21 §1001, et seq.

22 Sec. 4. Section 510B.8B, Code 2025, is amended to read as
23 follows:

24 **510B.8B Pharmacy benefits manager affiliates managers —**
25 **~~reimbursement~~ reimbursements.**

26 1. A pharmacy benefits manager shall not reimburse any
27 pharmacy located in the state in an amount less than the amount
28 that the pharmacy benefits manager reimburses a pharmacy
29 benefits manager affiliate for dispensing the same prescription
30 drug as dispensed by the pharmacy. ~~The reimbursement amount~~
31 ~~shall be calculated on a per unit basis based on the same~~
32 ~~generic product identifier or generic code number.~~

33 2. a. A pharmacy benefits manager shall not reimburse any
34 pharmacy located in the state in an amount less than the most
35 recently published national average drug acquisition cost for

1 a prescription drug on the date that the prescription drug is
2 administered or dispensed. If the most recently published
3 national average drug acquisition cost for the prescription
4 drug is unavailable on the date that the prescription drug is
5 administered or dispensed, a pharmacy benefits manager shall
6 not reimburse any pharmacy located in the state in an amount
7 less than the wholesale acquisition cost for the prescription
8 drug on the date that the prescription drug is administered or
9 dispensed.

10 b. In addition to the reimbursement under paragraph "a",
11 an agreement executed by a pharmacy benefits manager and a
12 pharmacy may, at the discretion of the pharmacy, require
13 the pharmacy benefits manager to reimburse the pharmacy
14 a professional dispensing fee for all prescription drugs
15 dispensed by the pharmacy in an amount not to exceed ten
16 dollars and sixty-eight cents if any of the following apply:

17 (1) The pharmacy has gross annual revenue of less than two
18 hundred fifty million dollars. For purposes of determining a
19 pharmacy's gross annual revenue under this subparagraph, the
20 gross revenue of all pharmacies under common ownership shall
21 be aggregated.

22 (2) The pharmacy has gross annual revenue of two hundred
23 fifty million dollars or greater and is located at least
24 ten miles away from the nearest pharmacy. For purposes of
25 determining a pharmacy's gross annual revenue under this
26 subparagraph, the gross revenue of all pharmacies under common
27 ownership shall be aggregated.

28 c. Paragraphs "a" and "b" do not apply to either of the
29 following:

30 (1) A specialty drug dispensed by a pharmacy located in this
31 state.

32 (2) A pharmacy located in this state that is a covered
33 entity as defined under 42 U.S.C. §256b.

34 3. Each pharmacy located in the state shall participate
35 in the national average drug acquisition cost survey for all

1 prescription drugs dispensed by the pharmacy.

2 Sec. 5. NEW SECTION. 510B.8D **Pharmacy benefits manager**
3 **contracts.**

4 1. All contracts executed, amended, adjusted, or renewed
5 on or after July 1, 2025, that apply to prescription drug
6 benefits on or after January 1, 2026, between a pharmacy
7 benefits manager and a third-party payor, or between a person
8 and a third-party payor, shall include all of the following
9 requirements:

10 a. The pharmacy benefits manager shall use pass-through
11 pricing unless paragraph "b" applies.

12 b. The pharmacy benefits manager may use direct or indirect
13 spread pricing only if the difference between the amount the
14 third-party payor pays the pharmacy benefits manager for a
15 prescription drug and the amount the pharmacy benefits manager
16 reimburses the dispensing pharmacy or dispensing health care
17 provider for the prescription drug is passed through by the
18 pharmacy benefits manager to the person contracted to receive
19 third-party payor services.

20 c. Payments received by a pharmacy benefits manager for
21 services provided by the pharmacy benefits manager to a
22 third-party payor or to a pharmacy shall be used or distributed
23 pursuant to the pharmacy benefits manager's contract with
24 the third-party payor or with the pharmacy, or as otherwise
25 required by law.

26 2. Unless otherwise prohibited by law, subsection 1 shall
27 supersede any contractual terms to the contrary in any contract
28 executed, amended, adjusted, or renewed on or after July 1,
29 2025, that applies to prescription drug benefits on or after
30 January 1, 2026, between a pharmacy benefits manager and a
31 third-party payor, or between a person and a third-party payor.

32 Sec. 6. NEW SECTION. 510B.8E **Appeals and disputes.**

33 1. A pharmacy benefits manager shall provide a reasonable
34 process to allow a pharmacy to appeal a reimbursement rate for
35 a specific prescription drug if the pharmacy benefits manager

1 violates section 510B.8A or 510B.8B.

2 2. The appeals process must include all of the following:

3 a. A dedicated telephone number at which a pharmacy may
4 contact the pharmacy benefits manager and speak directly with
5 an individual who is involved with the appeals process.

6 b. A dedicated electronic mail address or internet site for
7 the purpose of submitting an appeal directly to the pharmacy
8 benefits manager.

9 c. A period of no less than thirty business days after the
10 date of a pharmacy's initial submission of a clean claim during
11 which the pharmacy may initiate an appeal.

12 3. The pharmacy benefits manager shall respond to an appeal
13 within seven business days after the date on which the pharmacy
14 benefits manager receives the appeal.

15 a. If the pharmacy benefits manager grants a pharmacy's
16 appeal, the pharmacy benefits manager shall do all of the
17 following:

18 (1) Adjust the reimbursement rate of the prescription drug
19 that is the subject of the appeal and provide the national drug
20 code number that the adjustment is based on to the appealing
21 pharmacy.

22 (2) Reverse and resubmit the claim that is the subject of
23 the appeal.

24 b. If the pharmacy benefits manager denies a pharmacy's
25 appeal, the pharmacy may submit the denial to the commissioner
26 for examination.

27 Sec. 7. EFFECTIVE DATE. This division of this Act takes
28 effect January 1, 2026.

29 Sec. 8. APPLICABILITY. This division of this Act applies
30 to pharmacy benefits managers, health carriers, third-party
31 payors, and health benefit plans that manage a prescription
32 drug benefit in the state on or after July 1, 2025.

33 DIVISION II

34 PHARMACY SERVICES ADMINISTRATIVE ORGANIZATIONS AND WHOLESALE
35 DISTRIBUTION

1 Sec. 9. PHARMACY SERVICES ADMINISTRATIVE ORGANIZATIONS AND
2 WHOLESALE DISTRIBUTION OF PRESCRIPTION DRUGS — REPORT.

3 1. By January 1, 2026, the commissioner of insurance, or
4 the commissioner of insurance's designee, shall review pharmacy
5 services administrative organizations and the wholesale
6 distribution of prescription drugs, and submit a report to the
7 general assembly containing the commissioner's findings and
8 recommendations. The report shall include, at a minimum, all
9 of the following:

10 a. A description and analysis of the prescription drug
11 wholesale distribution supply chain, including the market
12 concentration for the wholesale distribution of prescription
13 drugs, margins in the wholesale distribution of prescription
14 drugs, and the competition in the wholesale distribution of
15 prescription drugs.

16 b. A description of the role that pharmacy services
17 administrative organizations serve in the prescription drug
18 supply chain.

19 c. A description and analysis of the relationships between
20 pharmacy services administrative organizations, prescription
21 drug wholesalers, and retail pharmacies, including but
22 not limited to standard contracting terms, fees charged to
23 pharmacies, and contractual restrictions and limitations
24 applicable to retail pharmacies.

25 2. a. The commissioner of insurance shall submit the report
26 under subsection 1 in a manner that does not publicly disclose
27 any of the following:

28 (1) The identity of a specific pharmacy services
29 administrative organization or prescription drug wholesaler.

30 (2) The price charged to a specific pharmacy for a specific
31 prescription drug.

32 b. Information provided by the commissioner under this
33 section that may reveal the identity of a specific pharmacy
34 services administrative organization or prescription drug
35 wholesaler, or the price charged to a specific pharmacy for a

1 specific prescription drug, shall be considered a confidential
2 record.

3 Sec. 10. EFFECTIVE DATE. This division of this Act takes
4 effect January 1, 2026.>

5 2. Title page, by striking line 3 and inserting
6 <organizations, and including effective date and applicability
7 provisions.>

GUSTOFF of Polk